

COUNSELING SITUATIONS

Pastoral Wisdom for Common Problems and Crises

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1. From Principles to People

Biblical Counseling established the foundations: listen before answering, distinguish sin from suffering, consider heart, body, and circumstances, apply Scripture in context, know the limits of pastoral competence, and protect people in danger. This companion volume asks what those principles look like when a pastor faces actual situations.

Cases rarely arrive neatly labeled. A pastor may hear, “My marriage is falling apart,” and discover grief, alcohol misuse, financial secrecy, anger, and depression in the same household. The first skill is therefore not naming the problem but understanding the person and deciding what needs attention first.

A useful triage asks four questions: Is anyone in immediate danger? Is there a medical, psychiatric, legal, or safeguarding issue requiring specialized help? What spiritual and relational issues can pastoral care address? What is the next faithful step? This keeps urgent matters from being buried beneath less urgent ones.

2. Grief and Bereavement

Grief is a normal response to significant loss. Christians grieve with hope, not without grief. Jesus wept, the Psalms lament, and the New Testament joins resurrection hope with sorrow.

Do not impose a rigid timetable or assume everyone passes through identical stages. Some people cry openly; others become quiet, busy, irritable, forgetful, or exhausted. Anniversaries, holidays, songs, places, and ordinary routines can renew grief unexpectedly.

Pastoral care often consists of presence, practical help, prayer, remembering the person who died, and allowing lament without rushing to explanation. Watch for inability to function, severe hopelessness, dangerous substance use, or suicidal thinking; these call for additional assessment and possibly professional care.

Case: A widower says, “If I really believed she is with the Lord, I would not still cry.” The pastor can correct the false conclusion gently. Christian hope does not erase love or loss. Tears and resurrection faith can occupy the same room.

3. Depression and Despair

Not every period of sadness is clinical depression, and a pastor should not diagnose beyond training. Yet persistent low mood, loss of interest, major changes in sleep or appetite, inability to function, profound hopelessness, or thoughts of death deserve careful attention.

Depression is not automatically evidence of weak faith. Biblical servants experienced profound discouragement. Elijah became exhausted and despondent; the psalmists sometimes spoke from the depths. Their experiences should not be used as modern diagnoses, but they do demonstrate that faithful people can suffer deeply.

Ask directly about suicide when warning signs appear: “Have you been thinking about killing yourself?” If there is intent, a plan, access to lethal means, or inability to remain safe, obtain emergency or qualified professional help and do not leave the person alone.

Pastoral care can continue alongside treatment. Prayer, Scripture, sleep, nutrition, movement, relationships, meaningful routine, and medical or psychological care are not rival kingdoms.

4. Anxiety, Panic, and Fear

Anxiety can range from ordinary concern to disabling panic. Begin by listening for triggers, bodily symptoms, avoidance, sleep problems, stimulant use, medical conditions, recent losses, and the person's interpretation of what is happening.

Do not tell a person having a panic attack simply to “have more faith.” Help slow the situation, reduce immediate stimulation, encourage steady breathing, and remind the person that intense sensations can pass. Recurrent or unexplained episodes merit medical or mental-health evaluation.

Biblical teaching about God's care can help anxious believers distinguish what they can responsibly do from what they cannot control. Prayer is not a magical technique for eliminating all symptoms; it is an act of trust within uncertainty.

Case: A student cannot sleep before examinations and spends hours imagining failure. Help separate preparation from rumination: create a realistic study plan, establish sleep boundaries, reduce stimulants, pray specifically, and challenge catastrophic assumptions.

5. Marriage Conflict

Marriage problems require careful listening to both spouses, preferably with opportunities for individual assessment when serious concerns arise. Do not assume that every conflict is fifty-fifty.

Common patterns include criticism and withdrawal, defensiveness, contempt, financial secrecy, sexual conflict, interference from extended family, unrealistic expectations, and unresolved injuries. Help the couple move from accusations about character to specific behaviors and requests.

Repentance should be concrete. “I am sorry you were hurt” is not the same as “I lied about the money; that was wrong; here is how I will restore transparency.” Forgiveness and rebuilding trust are related but not identical.

When coercion, threats, stalking, forced sex, strangulation, or physical violence is present, ordinary couples counseling can increase danger. Safety and specialized intervention take priority.

6. Parenting and Family Strain

Parents often seek a technique when the deeper need is wisdom. Children differ in temperament, maturity, ability, and circumstances. Discipline should teach rather than merely discharge parental anger.

Parents should distinguish childishness from deliberate defiance and avoid demanding adult capacities from children. Rules should be understandable, consequences reasonably connected to behavior, and discipline consistent enough to be predictable.

Adolescence requires a gradual movement from control toward responsible freedom. Parents who attempt to control every thought and friendship may obtain outward compliance while losing honest communication.

Serious self-harm, eating problems, substance use, abuse, dangerous aggression, or abrupt major behavioral change requires more than a parenting lecture. Bring appropriate professional help into the circle of care.

7. Sexual Sin and Pornography

Sexual sin thrives in secrecy, rationalization, access, and repeated habit. Shame alone rarely produces durable change. Biblical counsel combines moral clarity with grace and a concrete plan.

For pornography, identify when, where, and why the behavior occurs. Remove easy access, establish honest accountability, address deception, change routines that repeatedly lead to temptation, and examine the desires being served—escape, entitlement, loneliness, anger, novelty, or control.

A spouse injured by sexual betrayal should not be pressured to restore trust immediately. Confession is the beginning of rebuilding, not proof that trust has already been restored.

Sexual abuse is categorically different from consensual sexual sin. Never assign moral blame to a victim for being assaulted, and never treat abuse as a private sexual indiscretion that the church should quietly manage.

8. Alcohol, Drugs, and Other Addictions

Addiction affects body, choices, relationships, habits, and spiritual life. Good pastoral care avoids two errors: treating the person as morally helpless and treating physiological dependence as if it were merely stubbornness.

Ask about amount, frequency, loss of control, withdrawal symptoms, consequences, previous attempts to stop, other substances, and safety. Withdrawal from alcohol or some sedative drugs can be dangerous and requires medical guidance.

Recovery plans may include medical treatment, residential or outpatient programs, peer support, church community, accountability, financial safeguards, removal of access, and repair of damaged relationships.

Relapse is serious information. Instead of only demanding another promise, examine what happened before the relapse and strengthen the plan.

9. Anger, Violence, and Domestic Abuse

Anger is not identical to abuse. Abuse is a pattern of coercion, intimidation, violence, sexual coercion, stalking, threats, isolation, or control. A person can abuse another while appearing calm.

Interviewing an abused spouse in the presence of the suspected abuser may prevent honest disclosure. Do not reveal a victim's safety plan or location to the abusive partner.

Pastors must resist spiritual manipulation. Commands about submission, forgiveness, reconciliation, or keeping vows must not be used to require someone to remain in immediate danger. Civil authorities have a legitimate role in restraining wrongdoing.

Where children or vulnerable adults may be abused, know the reporting requirements of the jurisdiction. Do not promise secrecy that the law, safety, or protection of another person may make impossible.

10. Suicide and Self-Harm

Statements such as “Everyone would be better without me,” giving away possessions, sudden farewells, researching lethal methods, or escalating self-harm require attention. Direct questions are appropriate.

Ask whether the person has thoughts of suicide, whether there is a plan, whether means are available, and whether the person believes he or she can stay safe. Do not turn this into an interrogation; the purpose is protection.

High or imminent risk requires emergency or qualified crisis assistance. Stay with the person or ensure responsible supervision while help is obtained. Do not rely on a promise, a prayer, or a written “contract” as a substitute for safety measures.

After a crisis, pastoral ministry remains important. Shame, family disruption, spiritual questions, treatment adherence, loneliness, and rebuilding ordinary life may require long-term support.

11. Trauma and Abuse

Trauma can affect memory, sleep, attention, bodily reactions, trust, and the sense of safety. Pastors should avoid forcing detailed retelling merely to satisfy curiosity or to produce an emotional breakthrough.

Listen without demanding perfect chronology. Traumatic memories can be fragmented, but this does not mean every allegation is automatically accurate. Take reports seriously, protect safety, document carefully when appropriate, and allow trained investigators or clinicians to perform roles for which they are qualified.

Do not tell a victim that suffering occurred because of inadequate faith or that immediate reconciliation is required. The Psalms of lament and the suffering of Christ give the church language for pain without denying evil.

Recovery may include professional trauma treatment, medical care, legal protection, supportive relationships, stable routines, prayer, worship, and time.

12. Serious Mental Disturbance

Pastors sometimes encounter hallucinations, delusions, severe confusion, extreme agitation, mania, or dramatic personality changes. These symptoms can have psychiatric, neurological, medical, substance-related, or medication-related causes.

Do not assume unusual experiences are demonic, and do not assume spiritual questions are meaningless merely because illness is present. Begin with safety and appropriate medical or mental-health evaluation.

A person who has gone days with almost no sleep, speaks rapidly, makes reckless decisions, and develops grandiose plans may require urgent assessment. Likewise, sudden confusion in an older adult can signal a medical emergency rather than a counseling problem.

Pastoral ministry can support the person and family without pretending to provide a diagnosis. The church can reduce stigma, encourage treatment, provide practical help, and continue ordinary Christian fellowship.

13. Financial Crisis and Unemployment

Money problems can involve poverty, unemployment, illness, debt, gambling, overspending, secrecy, exploitation, or simple lack of financial knowledge. Do not assume that every poor person is irresponsible or every indebted person is merely a victim.

Begin with facts. What income exists? What debts and obligations exist? Are food, shelter, medicine, or utilities at risk? Is there hidden spending or financial abuse? A simple written budget often reveals what general discussion does not.

Church assistance should combine generosity with wisdom. Emergency help may be necessary, but repeated assistance without understanding the underlying problem can unintentionally maintain destructive patterns.

Unemployment also wounds identity and hope. Practical help with contacts, transportation, applications, training, and temporary needs can be as pastoral as a conversation.

14. Aging, Illness, and Caregiving

Aging can bring grief over strength, independence, friends, hearing, vision, memory, and social role. It can also bring wisdom, freedom, and new forms of ministry. Pastoral care should resist treating older adults as either helpless children or spiritual heroes who have no struggles.

Caregivers often experience exhaustion, guilt, resentment, and isolation. Encouraging rest or respite is not selfishness. Churches can provide practical support rather than merely telling caregivers to pray harder.

Sudden confusion, falls, medication problems, dehydration, infection, or abrupt personality changes in an older adult deserve medical attention. Do not assume they are simply normal aging.

End-of-life conversations can include fears, family reconciliation, practical planning, prayer, hope of resurrection, and the person's wishes for care. The pastor should provide spiritual counsel without practicing medicine.

15. Spiritual Doubt and Crisis of Faith

Doubt is not always rebellion. It can arise from intellectual questions, suffering, disappointment with God, hypocrisy in the church, unanswered prayer, moral struggle, or fear.

Listen for the actual question. Giving an argument for God's existence to someone wounded by church abuse may answer a question the person did not ask. Likewise, emotional reassurance alone may not help someone with a serious intellectual objection.

Scripture contains faithful people who asked difficult questions. The Psalms, Habakkuk, Job, Thomas, and others show that questions can be brought into the presence of God.

The counselor should distinguish honest doubt from a settled decision to reject truth, while refusing to manipulate either person. Patience, careful study, prayer, and trustworthy Christian community can create room for faith to mature.

16. Church Conflict and Wounded Believers

Church conflict can involve doctrine, personality, leadership style, money, gossip, power, misconduct, or simple misunderstanding. "Unity" does not mean suppressing legitimate concerns.

Matthew 18 is often cited, but not every situation begins as a private disagreement between equal parties. Criminal conduct, abuse, harassment, or allegations against powerful leaders may require safeguarding procedures and outside authorities rather than an informal private meeting.

For ordinary conflict, encourage accurate speech, direct communication, willingness to hear correction, confession without excuses, and refusal to recruit factions. Distinguish preferences from biblical commands.

A person harmed by a church may need time before trusting another congregation. The goal is not to defend the institution at all costs but to pursue truth, justice, reconciliation where possible, and healthy Christian community.

17. When the Counselor Is Out of Depth

Competence includes knowing its boundaries. A pastor can be an excellent spiritual counselor and still need help from physicians, psychologists, psychiatrists, social workers, lawyers, addiction specialists, or safeguarding professionals.

Develop a referral network before a crisis. Learn who in the community is trustworthy, affordable, accessible, and respectful of the person's beliefs. In areas with limited resources, telehealth, public clinics, denominational networks, and nonprofit services may broaden options.

When referring, do not simply dismiss the person with “You need a professional.” Explain why additional expertise is useful and, with permission, remain pastorally involved.

Referral is collaboration, not abandonment.

18. A Pastoral Decision Guide

When a situation arrives, first ask about immediate safety. If there is danger, act on safety before extended counseling. Second, identify whether medical, psychiatric, legal, or safeguarding expertise is needed. Third, listen to the person's full story rather than treating the first complaint as the whole problem.

Fourth, distinguish suffering, sin, bodily factors, relationships, and circumstances. Fifth, identify the biblical truth that actually addresses the issue. Sixth, choose one or two realistic next steps. Seventh, decide who should follow up and when.

Good pastoral counseling does not require the pastor to solve every problem. It requires truthful listening, biblical wisdom, compassionate action, appropriate boundaries, and faithful follow-through.

The question is not, “Can I fix this person?” The better question is, “What does faithful love require next?”

Conclusion: The Next Faithful Step

People seldom come to a pastor at the neat beginning of a problem. They arrive in the middle of grief, conflict, fear, temptation, illness, confusion, or crisis. The counselor's task is to enter that confusion without becoming confused about the mission.

Listen carefully. Protect life. Tell the truth. Apply Scripture faithfully. Use the wisdom God provides through the church and through competent helpers. Do not promise what you cannot deliver, and do not abandon people because their problems are complicated.

Often the most useful question in pastoral care is also the simplest: What is the next faithful step?